

Module 5 — Your Pattern Map: From Guessing to Better Questions

Learning Objective

By the end of this module, you will be able to create a short, neutral pattern map for a recent non-emergency concern; distinguish observations from interpretations; and use a concise summary to support an appropriate discussion with a clinician when needed. A pattern map is a tool for noticing and communicating. It does not diagnose a condition or establish a “root cause.”

Three Key Ideas

1. Separate signal, context, and interpretation

A **signal** is the direct experience you noticed, such as “I woke several times,” “I had a headache after lunch,” or “I felt sleepy during a meeting.” A **context** is what was happening around it, such as your schedule, travel, illness, room, stress, activity, caffeine, or medication timing as prescribed. An **interpretation** is a possible explanation, such as “the room caused it” or “my sleep score explains it.” Interpretations can generate useful questions, but they are not conclusions.

Use neutral language: “noticed,” “occurred alongside,” “recorded,” and “improved while away.” This keeps the map accurate enough to be useful and open enough to allow for multiple explanations.

2. A small, time-bounded map is more useful than a flood of data

For many concerns, a one- to two-week diary can provide practical context. The National Heart, Lung, and Blood Institute notes that a sleep diary may help a healthcare professional understand sleep timing and activities that can affect sleep; useful entries can include bedtime, wake time, naps, daytime sleepiness, caffeine or alcohol use, and exercise.¹

Your map does not need to capture everything. Record the concern, its timing, a few relevant contexts, its impact on daily function or safety, and the source of your information. “Memory,” “diary,” and “device estimate” are all valid source labels. Note what you do not know. Missing data are more honest—and more useful—than invented certainty.

3. Personal data can support questions; it does not deliver a verdict

Consumer wearables, apps, and home sensors may help you notice broad timing or behavior patterns, but their data can be incomplete, algorithmically inferred, altered by software updates,

or difficult to compare across devices.² A consumer sleep score or displayed sleep stage is not equivalent to a clinical sleep study and should not be used to diagnose or rule out a sleep disorder.³

The useful shift is from guessing to **appropriate personal data and an appropriate clinical discussion**. A clinician may use symptoms, health history, and examination, and may decide that a diary, laboratory test, sleep study, actigraphy, or no testing is appropriate for a particular question.¹ A test can inform a decision without explaining every symptom. Bring a focused summary rather than a large data dump.

Notice This Week: A One-Page Pattern Map

Choose one recent, non-emergency concern. Use a paper notebook or a simple private note. Take no more than two minutes a day. If tracking increases worry, becomes compulsive, or does not feel useful, pause or stop.

Date/time	What I noticed (observable)	Context or co-occurrences	Function or safety impact	Source and confidence	Question, not conclusion
				Memory / diary / device estimate	
				Memory / diary / device estimate	
				Memory / diary / device estimate	
				Memory / diary / device estimate	
				Memory / diary / device estimate	

End-of-Week Summary

Complete this sentence in your own words:

From [date range], I noticed [experience] about [frequency/timing], most often alongside [contexts]. The impact on daily function or safety was [impact]. My information comes from [source] and is limited by [missingness, memory, or device limitations]. I would like to understand what possibilities should be considered and whether any evaluation is appropriate.

Before you review the week, remember three safeguards. Co-occurrence does not prove causation. Memory and attention can be selective, especially when you are worried. One person's observations do not establish a general rule.

Low-Risk Experiment: Seven Days of Observation Without Changing the Plan

For seven days, observe one concern while keeping your usual routine as stable as reasonably possible. Do not use this activity to start, stop, or change prescribed medication, supplements, diet, devices, or environmental remediation. Record one brief entry each day using the table above.

At the end of the week, compare repeated episodes rather than focusing on the most dramatic day. You might compare weekdays and weekends, usual and unusual schedules, or different locations. If you use a wearable, record its name, the date range, and whether the displayed value is measured or inferred. Do not repeatedly check scores, and do not upload health or household data to the course. Review app permissions, cloud sharing, and who can access exports.

The intended result is not proof. The result is one clearer question—or the realization that you need less tracking, not more.

Three Questions for an Appropriate Clinician or Testing Discussion

1. "I am bringing a pattern summary, not a diagnosis. What explanations should we consider, and which details would change your evaluation?"
2. "Based on my symptoms, history, and functional impact, would a diary, examination, laboratory test, sleep study, or another step be appropriate—and what specific

question would it address?”

3. **“How should I interpret a normal, abnormal, or inconclusive result, and what symptoms or changes should prompt earlier follow-up?”**

Bring a short, relevant date range and a summary sentence. Share medication and substance information accurately, but do not make treatment changes based on the map. Ask whether selected device data would be helpful, or whether no further tracking is preferable.

Red-Flag and Escalation Note

Seek timely guidance from an appropriate healthcare professional for symptoms that are persistent, recurrent, worsening, or meaningfully affecting daily function. Regular sleep difficulty, repeated awakenings, and daytime sleepiness or tiredness are reasons to discuss sleep concerns with a provider.⁴ If sleepiness could impair driving or operating machinery, reduce the immediate risk and do not drive when drowsy.

This worksheet is not for emergency triage. Seek urgent or emergency care for severe breathing difficulty, chest pain, fainting, sudden neurologic changes, severe allergic reaction, poisoning concern, or other emergency symptoms. If you suspect an environmental hazard, avoid unsafe do-it-yourself inspection or remediation; seek qualified local help and relevant public-health guidance.

Educational Disclaimer

This handout provides educational wellness information only. It is not medical advice, diagnosis, treatment, prevention, or a substitute for care from a licensed clinician. A pattern map, consumer device reading, home sensor, or single test does not establish a universal cause of a symptom. Do not start, stop, or change medications, supplements, diet, devices, or environmental actions based solely on this handout or your observations.

References

-
1. National Heart, Lung, and Blood Institute. “Insomnia Diagnosis.” <https://www.nhlbi.nih.gov/health/insomnia/diagnosis>
 2. de Zambotti, M., Cellini, N., Goldstone, A., Colrain, I. M., & Baker, F. C. (2019). “Wearable Sleep Technology in Clinical and Research Settings.” *Medicine & Science in Sports & Exercise*. <https://pmc.ncbi.nlm.nih.gov/articles/PMC6579636/>

3. American Academy of Sleep Medicine. "Consumer Sleep Technology: An American Academy of Sleep Medicine Position Statement." <https://aasm.org/advocacy/position-statements/consumer-sleep-technology/>
4. Centers for Disease Control and Prevention. "About Sleep." <https://www.cdc.gov/sleep/about/index.html>